

PRIOR AUTHORIZATION REQUEST

Cover Letter

Patient Name	John Smith
Date of Birth	1968-03-15
Insurance	United Healthcare
Member ID	UHC-887612
Policy Number	UHC-POL-2026-001
CPT Code	75563
ICD-10	I42.0
Physician	Dr. Ramesh Kumar
Physician NPI	1234567890
Facility	City Heart Institute
Date of Request	March 26, 2026

United Healthcare Prior Authorization Department

Radiology Prior Authorization Team

[Payer Address - if known, otherwise omitted as per instructions]

March 26, 2026

****RE: Prior Authorization Request for Cardiac MRI with Contrast (CPT 75563)****

****Patient:** John Smith**

****Date of Birth:** 1968-03-15**

****Insurance Member ID:** UHC-887612**

****Requesting Physician:** Dr. Ramesh Kumar**

****NPI:** 1234567890**

****Specialty:** Cardiology**

****Facility:** City Heart Institute**

Dear Prior Authorization Review Team,

This letter serves as a formal request for prior authorization for a Cardiac MRI with Contrast (CPT 75563) for our patient, Mr. John Smith.

Mr. Smith, a 58-year-old male, has a documented 3-year history of Dilated Cardiomyopathy (ICD-10: I42.0). Despite strict adherence to optimal Guideline-Directed Medical Therapy (GDMT) for this duration, including

Carvedilol 25mg twice daily, Lisinopril 10mg daily, Spironolactone 25mg daily, and Furosemide 40mg daily, his clinical condition has shown inadequate improvement. He continues to experience significant symptoms consistent with New York Heart Association (NYHA) Class III heart failure, including worsening shortness of breath on exertion, persistent fatigue, and a marked reduction in exercise tolerance. Recent diagnostic findings further underscore the severity of his condition: a transthoracic echocardiogram performed on January 15, 2026, documented a severely reduced Left Ventricular Ejection Fraction (LVEF) of 35% with diffuse global hypokinesis and a moderately dilated left ventricle. Furthermore, recent lab results from March 10, 2026, show a significantly elevated BNP level of 450 pg/mL, consistent with moderate to severe heart failure. While echocardiography provides a functional assessment, it is insufficient for definitive myocardial tissue characterization, assessment of myocardial fibrosis via Late Gadolinium Enhancement (LGE), or differentiation between ischemic and non-ischemic cardiomyopathy. The requested Cardiac MRI with contrast is medically necessary to provide high-resolution data crucial for assessing myocardial viability, guiding critical treatment decisions such as potential revascularization or implantable cardioverter-defibrillator (ICD) placement, and ultimately improving Mr. Smith's prognosis and quality of life.

We have attached all supporting clinical documentation, including the Clinical Justification Note, Echocardiogram Report, Recent Lab Results, Physician Order Letter, and Prior Treatment Records, for your comprehensive review.

Thank you for your prompt attention to this urgent request.

Sincerely,

Prior Authorization Specialist

On behalf of Dr. Ramesh Kumar

NPI: 1234567890

Cardiology

City Heart Institute

Clinical Summary

Clinical Summary for Prior Authorization

1. Diagnosis

ICD-10: I42.0 - Dilated Cardiomyopathy

2. Patient History

Mr. John Smith, a 58-year-old male, has a 3-year history of Dilated Cardiomyopathy (diagnosed Jan 2023) with a chronic, progressive course. He presents with New York Heart Association (NYHA) Class III heart failure symptoms, including worsening dyspnea on exertion, persistent fatigue, reduced exercise tolerance, and occasional palpitations. Comorbidities include mild functional mitral and tricuspid regurgitation.

3. Reason for Procedure (CPT 75563 - Cardiac MRI with Contrast)

Cardiac MRI with contrast (CPT 75563) is requested for definitive myocardial tissue characterization. Despite 3 years of optimal guideline-directed medical therapy (GDMT), Mr. Smith's symptoms have progressed, and his Left Ventricular Ejection Fraction (LVEF) remains severely reduced. A recent echocardiogram (Jan 15, 2026) was insufficient to fully assess myocardial viability, specific fibrosis patterns, or differentiate cardiomyopathy etiology. This advanced imaging is crucial to guide future treatment decisions, including potential revascularization or implantable cardioverter-defibrillator (ICD) placement.

4. Supporting Evidence

* **Echocardiogram (Jan 15, 2026):** Documented LVEF of 35% with diffuse global hypokinesis and a moderately dilated left ventricle (LV End Diastolic Diameter 6.2 cm). The report recommended Cardiac MRI for further tissue characterization.

* **Lab Results (March 10, 2026):** Elevated BNP at 450 pg/mL (reference < 100 pg/mL) and NT-proBNP at 1850 pg/mL (reference < 125 pg/mL for age < 75), consistent with moderate to severe heart failure. Renal function is preserved (Creatinine 1.1 mg/dL, eGFR 72 mL/min).

* **Clinical Status:** Patient exhibits NYHA Class III symptoms, including shortness of breath at rest/minimal exertion, orthopnea requiring 3-pillow support, and bilateral lower extremity edema.

* **Prior Treatments:** Patient has been strictly compliant with GDMT for 3 years, including Carvedilol 25mg BID, Lisinopril 10mg daily, Spironolactone 25mg daily, and Furosemide 40mg daily. Sacubitril/Valsartan and Ivabradine were trialed but discontinued due to adverse effects (hypotension/dizziness) and inadequate response, respectively. Symptoms persist despite maximal tolerated GDMT.

5. Clinical Justification

Cardiac MRI with contrast is the gold standard for comprehensive myocardial tissue characterization, including assessment of myocardial fibrosis via Late Gadolinium Enhancement (LGE) and evaluation of myocardial viability. This procedure is medically necessary to differentiate cardiomyopathy etiology (ischemic vs. non-ischemic), assess prognosis, and directly impact the clinical treatment plan by informing decisions regarding revascularization, advanced heart failure therapies, or ICD implantation, aligning with current cardiology guidelines for patients with progressive heart failure and reduced LVEF despite optimal GDMT.

Requirements Checklist

Requirement	Status	Evidence
Clinical Justification Note: Detailed note from treating Cardiologist explaining medical necessity, including patient history, current symptoms, previous treatments tried, reason echocardiogram is insufficient. Must be signed and dated.	✓ MET	The 'Clinical Justification Note' from Dr. Ramesh Kumar MD, dated March 16, 2026, is uploaded. It details a 3-year history of Dilated Cardiomyopathy, current NYHA Class III symptoms (worsening SOB, fatigue, reduced exercise tolerance, palpitations), lists 3+ years of Guideline-Directed Medical Therapy (GDMT) including Carvedilol, Lisinopril, Spironolactone, Furosemide, and explains echocardiogram insufficiency for tissue characterization and viability. The note is signed and dated.
Echocardiogram Report: Most recent echocardiogram report within last 12 months. Must document LVEF value, chamber dimensions, wall motion analysis, and valve function.	✓ MET	The 'Echocardiogram Report' dated January 15, 2026 (within 12 months of March 26, 2026), is uploaded. It documents LVEF of 35%, LV End Diastolic Diameter 6.2 cm, LV End Systolic Diameter 5.1 cm, diffuse global hypokinesis, and details on mitral, aortic, tricuspid, and pulmonic valve function.
Recent Lab Results: Laboratory results within last 30 days including: BNP or NT-proBNP levels, complete metabolic panel, complete blood count.	✓ MET	The 'Laboratory Results Report' dated March 10, 2026 (within 30 days of March 26, 2026), is uploaded. It includes BNP 450 pg/mL, NT-proBNP 1850 pg/mL, a complete blood count (WBC, RBC, Hemoglobin, Hematocrit, Platelets, MCV), and a comprehensive metabolic panel (Sodium, Potassium, Creatinine, eGFR, BUN, Glucose, AST, ALT, Albumin).
Physician Order Letter: Official order on physician letterhead signed by referring Cardiologist. Must include: procedure requested, ICD-10 diagnosis code, clinical indication, physician NPI number.	✓ MET	The 'Physician Order Letter' on City Heart Institute letterhead, signed by Dr. Ramesh Kumar MD, dated March 16, 2026, is uploaded. It specifies 'CARDIAC MRI WITH CONTRAST (CPT 75563)', ICD-10 code I42.0, clinical indications (tissue characterization, fibrosis, viability, differentiation of ischemic/non-ischemic), and physician NPI number 1234567890.
Prior Treatment Records: Documentation of cardiac medications tried for minimum 3 months including drug names, dosages, duration of therapy, and patient response or reason for discontinuation.	✓ MET	The 'Prior Treatment Records' dated March 16, 2026, is uploaded. It details Carvedilol, Lisinopril, Spironolactone, and Furosemide with durations ranging from 2 years 9 months to 3 years, including dosages, patient response, and current status. It also lists discontinued Sacubitril/Valsartan and Ivabradine with reasons for discontinuation.

Eligibility Criteria: Patient must have a confirmed diagnosis of Dilated Cardiomyopathy (ICD-10: I42.0), Hypertrophic Cardiomyopathy (ICD-10: I42.1), Restrictive Cardiomyopathy (ICD-10: I42.5), Cardiac Sarcoidosis (ICD-10: D86.85), or Suspected Cardiac Tumor or Mass.	✓ MET	The patient's primary diagnosis is 'Dilated Cardiomyopathy (ICD-10: I42.0)' as stated in the EHR data, Clinical Justification Note, and Physician Order Letter.
Eligibility Criteria: Symptoms (Shortness of breath on exertion, Chest pain or pressure, Palpitations or arrhythmia, Unexplained syncope or near-syncope, Reduced exercise tolerance) must be present and documented for a minimum period of 3 months.	✓ MET	The Clinical Justification Note documents 'worsening shortness of breath on exertion, persistent fatigue, and a marked reduction in exercise tolerance... occasional palpitations' with a '3-year history of Dilated Cardiomyopathy' and 'chronic and progressive clinical course.' Prior Treatment Records confirm 'NYHA Class III symptoms' and 'Reduced functional capacity,' indicating symptoms present for well over 3 months.
Eligibility Criteria: Patient must have completed an echocardiogram prior to the MRI request.	✓ MET	An 'Echocardiogram Report' dated January 15, 2026, was completed prior to the Cardiac MRI request date of March 16, 2026.
Eligibility Criteria: The echocardiogram must show a documented LVEF (Left Ventricular Ejection Fraction) value and evidence of wall motion abnormality or structural cardiac abnormality requiring further characterization.	✓ MET	The 'Echocardiogram Report' dated January 15, 2026, documents an LVEF of 35%, 'Diffuse global hypokinesis' (wall motion abnormality), and a 'Moderately dilated left ventricle' (structural cardiac abnormality). The report concludes with a recommendation for 'Cardiac MRI for further tissue characterization.'
Eligibility Criteria: The request must be ordered by a board-certified Cardiologist or Cardiac Electrophysiologist.	✓ MET	Dr. Ramesh Kumar MD, the ordering physician, is a 'Board Certified Cardiologist' as confirmed by the EHR data, Clinical Justification Note, and Physician Order Letter.
Eligibility Criteria: Cardiac MRI is covered when echocardiogram findings are inconclusive.	✓ MET	The Clinical Justification Note states the echocardiogram 'is insufficient to fully characterize the myocardial tissue or identify specific fibrosis patterns,' and 'lacks the resolution required to assess myocardial viability or definitively rule out infiltrative cardiomyopathies.' The Echocardiogram Report also recommends 'Cardiac MRI for further tissue characterization.'
Eligibility Criteria: Cardiac MRI is covered for assessment of myocardial viability.	✓ MET	The Clinical Justification Note states the MRI is 'critical to assess myocardial viability for potential revascularization candidacy.' The Physician Order Letter also lists 'Assessment of myocardial viability' as a medical necessity.
Eligibility Criteria: Cardiac MRI is covered for evaluation of cardiac masses or tumors.	✗ MISSING	The EHR data and supporting documents do not indicate that the Cardiac MRI is being requested for the evaluation of cardiac masses or tumors. The primary indications are related to Dilated Cardiomyopathy, tissue characterization, fibrosis, and viability.

Eligibility Criteria: Cardiac MRI is covered for assessment of pericardial disease.	✗ MISSING	While the Echocardiogram Report notes a 'Small pericardial effusion,' the Clinical Justification Note and Physician Order Letter do not list assessment of pericardial disease as a primary clinical indication for the Cardiac MRI. The MRI is requested for myocardial characterization related to DCM.
Eligibility Criteria: Cardiac MRI is covered for congenital heart disease evaluation.	✗ MISSING	The patient's diagnosis is Dilated Cardiomyopathy, not congenital heart disease. There is no evidence in the EHR to support this indication.
Eligibility Criteria: Repeat MRI within 6 months is not covered without new symptoms.	✓ MET	There is no documentation of a previous Cardiac MRI within the last 6 months in the provided EHR, indicating this is either an initial request or a repeat outside the 6-month window. If it were a repeat, new symptoms (worsening SOB, fatigue, palpitations, reduced exercise tolerance) are documented in the Clinical Justification Note.

Attached Documents

The following documents are attached to this package:

#		
1	Clinical Justification Note Kumar	Clinical_Justification_Note_Kumar.pdf
2	Echocardiogram Report Jan2026	Echocardiogram_Report_Jan2026.pdf
3	Laboratory Results Report	Laboratory_Results_Report.pdf
4	Physician Order Prior Auth Cardiac Mri	Physician_Order_Prior_Auth_Cardiac_MRI.pdf
5	Prior Treatment Records Cardiac Medication	Prior_Treatment_Records_Cardiac_Medication.pdf

Physician: Dr. Ramesh Kumar MD

NPI Number: 1234567890

Date: March 16, 2026

RE: PRIOR AUTHORIZATION REQUEST FOR CARDIAC MRI WITH CONTRAST (CPT 75563)

PATIENT HISTORY

The patient is a 58-year-old male with a documented 3-year history of Dilated Cardiomyopathy (ICD-10: I42.0). Initially diagnosed in January 2023 following a presentation with acute dyspnea, the patient has since exhibited a chronic and progressive clinical course. Current presentation is significant for worsening shortness of breath on exertion, persistent fatigue, and a marked reduction in exercise tolerance that interferes with activities of daily living. Furthermore, the patient reports occasional palpitations, necessitating advanced diagnostic evaluation to assess for structural triggers or arrhythmogenic substrates.

CURRENT SYMPTOMS

At present, the patient exhibits symptoms consistent with New York Heart Association (NYHA) Class III heart failure. Clinical findings include shortness of breath both at rest and upon minimal physical exertion, as well as significant orthopnea requiring 3-pillow support for sleep. Physical examination reveals bilateral lower extremity edema, indicating volume overload. Recent diagnostic monitoring confirms a persistently reduced ejection fraction, signifying a lack of improvement despite standard therapeutic interventions.

PREVIOUS TREATMENTS TRIED

The patient has remained strictly compliant with Guideline-Directed Medical Therapy (GDMT) since diagnosis. The therapeutic regimen includes the following:

- Carvedilol: 25mg twice daily (initiated January 2023)
- Lisinopril: 10mg daily (initiated January 2023)
- Spironolactone: 25mg daily (initiated March 2023)
- Furosemide: 40mg daily for fluid management

Despite adherence to optimal medical therapy for over three years, the patient's symptoms persist and have shown recent signs of progression, suggesting the underlying pathology requires more granular characterization than what is provided by current management tools.

CLINICAL INSUFFICIENCY OF ECHOCARDIOGRAPHY

A transthoracic echocardiogram (TTE) performed in January 2026 documented a Left Ventricular Ejection Fraction (LVEF) of 35% with global hypokinesis. While the TTE provided a baseline assessment of chamber size and function, it is insufficient to fully characterize the myocardial tissue or identify specific fibrosis patterns. Standard echocardiography lacks the resolution required to assess myocardial viability or definitively rule out infiltrative cardiomyopathies. Consequently, a Cardiac MRI with contrast is essential for

definitive tissue characterization and to provide the high-resolution data necessary to guide future high-stakes treatment decisions.

MEDICAL NECESSITY STATEMENT

Cardiac MRI with contrast is medically necessary for this patient to evaluate myocardial fibrosis via Late Gadolinium Enhancement (LGE). This study is critical to assess myocardial viability for potential revascularization candidacy and to differentiate between ischemic and non-ischemic cardiomyopathy. The findings from this advanced imaging will directly impact the clinical treatment plan, influence prognosis, and determine the appropriateness of further interventional therapies.

Respectfully,

Dr. Ramesh Kumar MD
Date Signed: March 16, 2026
Credentials: Board Certified Cardiologist
License Number: MD-12345-TX

TRANSTHORACIC ECHOCARDIOGRAM REPORT

Ordering Physician:

Dr. Ramesh Kumar MD

Performing Physician:

Dr. Sarah Johnson MD

Facility:

City Heart Institute

Clinical Indication:

Dilated Cardiomyopathy; Follow up evaluation

Study Date:

January 15, 2026

Report Date:

January 15, 2026

Study Type:

Complete TTE

SECTION 1: TECHNICAL QUALITY

Study Quality: Adequate

Windows: Acceptable acoustic windows obtained.

Views: Parasternal long axis, short axis, apical 4-chamber, 2-chamber, 3-chamber, and subcostal views were successfully obtained and analyzed.

SECTION 2: LEFT VENTRICLE MEASUREMENTS

Measurement	Result	Normal Reference Range
LV End Diastolic Diameter	6.2 cm	Less than 5.5
LV End Systolic Diameter	5.1 cm	Less than 3.9
Interventricular Septum	0.8 cm	0.6 to 1.1
Posterior Wall Thickness	0.8 cm	0.6 to 1.1
LV Ejection Fraction (LVEF)	35%	Greater than 55%
LV Fractional Shortening	18%	25 to 45%

SECTION 3: LEFT VENTRICLE FUNCTION

LVEF: 35% – Severely reduced.

Wall Motion: Diffuse global hypokinesis noted. No regional wall motion abnormalities (RWMA) identified.

Diastolic Function: Grade II diastolic dysfunction (pseudonormal pattern).

LV Size: Moderately dilated left ventricle.

SECTION 4: RIGHT VENTRICLE

RV Size: Normal.

RV Function: Mildly reduced.

TAPSE: 1.6 cm (mildly reduced).

RV Systolic Pressure: Estimated 38 mmHg.

SECTION 5: VALVES

Mitral Valve: Mild mitral regurgitation identified. Etiology appears functional, secondary to LV dilation and annular stretching.

Aortic Valve: Trileaflet, normal opening. No significant stenosis or regurgitation visualized.

Tricuspid Valve: Mild tricuspid regurgitation.

Pulmonic Valve: Normal appearance and function.

SECTION 6: OTHER FINDINGS

Pericardium: Small pericardial effusion noted.

Aorta: Normal aortic root diameter.

IVC: Dilated IVC suggesting elevated right atrial pressure.

SECTION 7: CONCLUSION

1. Severely reduced left ventricular systolic function with LVEF of 35%.
2. Moderately dilated left ventricle consistent with dilated cardiomyopathy.
3. Diffuse global hypokinesis.
4. Mild mitral regurgitation — functional etiology.
5. Grade II diastolic dysfunction.
6. Small pericardial effusion.
7. Recommend Cardiac MRI for further tissue characterization.

Interpreting Physician: Dr. Sarah Johnson MD

Board Certified Echocardiographer

Date: January 15, 2026

Ordering Physician:	Dr. Ramesh Kumar MD	Specimen Type:	Venous Blood
Collection Date:	March 10, 2026	Clinical Indication:	Dilated Cardiomyopathy
Report Date:	March 10, 2026		Cardiac MRI Pre-authorization

SECTION 1 - CARDIAC BIOMARKERS

Test	Result	Unit	Reference Range	Flag
BNP (B-type Natriuretic Peptide)	450	pg/mL	Less than 100	HIGH
NT-proBNP	1850	pg/mL	Less than 125 (age < 75)	HIGH
Troponin I (High Sensitivity)	0.04	ng/mL	Less than 0.04	BORDERLINE

SECTION 2 - COMPLETE BLOOD COUNT

Test	Result	Unit	Reference Range	Flag
WBC	7.2	10^3/uL	4.5 to 11.0	NORMAL
RBC	4.8	10^6/uL	4.5 to 5.9	NORMAL
Hemoglobin	13.8	g/dL	13.5 to 17.5	NORMAL
Hematocrit	41.2	%	41 to 53	NORMAL
Platelets	215	10^3/uL	150 to 400	NORMAL
MCV	88	fL	80 to 100	NORMAL

SECTION 3 - COMPREHENSIVE METABOLIC PANEL

Test	Result	Unit	Reference Range	Flag
Sodium	138	mEq/L	136 to 145	NORMAL
Potassium	4.1	mEq/L	3.5 to 5.1	NORMAL
Creatinine	1.1	mg/dL	0.7 to 1.3	NORMAL
eGFR	72	mL/min	Greater than 60	NORMAL
BUN	18	mg/dL	7 to 20	NORMAL

Test	Result	Unit	Reference Range	Flag
Glucose	95	mg/dL	70 to 100	NORMAL
AST	28	U/L	10 to 40	NORMAL
ALT	22	U/L	7 to 56	NORMAL
Albumin	3.8	g/dL	3.5 to 5.0	NORMAL

SECTION 4 - LIPID PANEL

Test	Result	Unit	Reference Range	Flag
Total Cholesterol	195	mg/dL	Less than 200	NORMAL
LDL	118	mg/dL	Less than 130	NORMAL
HDL	42	mg/dL	Greater than 40	NORMAL
Triglycerides	175	mg/dL	Less than 150	BORDERLINE HIGH

SECTION 5 - INTERPRETATION NOTE

Significantly elevated BNP at 450 pg/mL and NT-proBNP at 1850 pg/mL consistent with moderate to severe heart failure.
Renal function preserved. Electrolytes within normal limits.
Results support ongoing heart failure management and prior authorization for advanced cardiac imaging.

Authorized By:

Dr. Michael Chen MD
Board Certified Pathologist
Date: March 10, 2026

CITY HEART INSTITUTE

Department of Cardiology | 123 Medical Center Drive | Houston, Texas 77001

Phone: 555-123-4567 | Fax: 555-123-4568 | www.cityheartinstitute.com

Date: March 16, 2026

To:

United Healthcare Prior Authorization Department

Radiology Prior Authorization Team

From:

Dr. Ramesh Kumar MD

Board Certified Cardiologist

NPI: 1234567890 | License: MD-12345-TX | DEA: RK1234563

SUBJECT: PRIOR AUTHORIZATION REQUEST

PROCEDURE: CARDIAC MRI WITH CONTRAST (CPT 75563)

ICD-10 CODE: I42.0

REQUESTED PROCEDURE

Name:	Cardiac MRI with Contrast
CPT Code:	75563
Urgency:	Routine — Non Emergency
Facility:	City Heart Institute Radiology
Estimated Date:	Within 30 days of approval

CLINICAL INDICATION

Primary Diagnosis: Dilated Cardiomyopathy (ICD-10: I42.0)

Secondary Diagnosis: Heart Failure with Reduced Ejection Fraction (ICD-10: I50.20)

MEDICAL NECESSITY STATEMENT

This procedure is medically necessary for:

- Tissue characterization of myocardium
- Assessment of myocardial fibrosis via late gadolinium enhancement imaging
- Differentiation of ischemic versus non-ischemic cardiomyopathy
- Assessment of myocardial viability
- Guidance of treatment decisions including consideration of ICD implantation

SUPPORTING CLINICAL INFORMATION

- LVEF 35% on recent echocardiogram
- Elevated BNP 450 pg/mL
- NYHA Class III symptoms
- Optimal medical therapy ongoing
- No contraindications to MRI or gadolinium contrast media

I certify that this procedure is medically necessary and appropriate for this patient. All information provided is accurate and complete to the best of my knowledge.

Dr. Ramesh Kumar MD
Board Certified Cardiologist
Date: March 16, 2026
NPI: 1234567890
Contact: 555-123-4567

Treating Physician:	Dr. Ramesh Kumar MD	Report Date:	March 16, 2026
Facility:	City Heart Institute	Specialty:	Cardiology
Period Covered:	Jan 2023 – Mar 2026	Clinical Indication:	Dilated Cardiomyopathy

This document summarizes the complete cardiac medication history for a patient with confirmed Dilated Cardiomyopathy (ICD-10: I42.0) over a period of 3 years from January 2023 to March 2026. All medications listed below were prescribed and monitored by the treating cardiologist as part of optimal medical therapy for heart failure with reduced ejection fraction.

SECTION 1 - MEDICATION HISTORY TABLE

Medication	Dose/ Freq	Dates	Duration	Response	Status
Carvedilol (Beta Blocker)	25mg Twice daily	Jan 2023 – Present	3 years	Partial response – HR controlled but symptoms persist	CURRENT
Lisinopril (ACE Inhibitor)	10mg Once daily	Jan 2023 – Present	3 years	Partial response – BP controlled but LVEF unchanged	CURRENT
Spirolactone (Aldo. Antagonist)	25mg Once daily	Mar 2023 – Present	2y 9mo	Minimal improvement in symptoms	CURRENT
Furosemide (Loop Diuretic)	40mg Once daily	Mar 2023 – Present	2y 9mo	Adequate fluid control; edema managed	CURRENT
Sacubitril/ Valsartan (ARNI)	24/26mg Twice daily	Sep 2023 – Nov 2023	2 months	Discontinued due to symptomatic hypotension and dizziness	DISCONTINUED
Ivabradine	5mg Twice daily	Jan 2024 – Mar 2024	2 months	Discontinued – inadequate heart rate reduction	DISCONTINUED

SECTION 2 - TREATMENT RESPONSE SUMMARY

Despite 3 years of optimal medical therapy with maximum tolerated doses of guideline directed medical therapy (GDMT) including beta blocker, ACE inhibitor, and aldosterone antagonist, patient continues to exhibit:

- Persistent LVEF of 35%
- NYHA Class III symptoms (marked limitation in activity)
- Elevated BNP levels (refer to lab report dated 03/10/2026)
- Reduced functional capacity

SECTION 3 - CURRENT MEDICATION LIST

- **Carvedilol:** 25mg twice daily
- **Lisinopril:** 10mg once daily
- **Spironolactone:** 25mg once daily
- **Furosemide:** 40mg once daily
- **Aspirin:** 81mg once daily
- **Atorvastatin:** 40mg once daily

Physician Attestation:

I certify that the above medication history is accurate and complete. The patient has received optimal guideline directed medical therapy for a minimum of 3 years with inadequate clinical response. Advanced cardiac imaging (Cardiac MRI) is required to guide further treatment decisions, including viability assessment and potential ICD placement.

Dr. Ramesh Kumar MD

Board Certified Cardiologist | NPI: 1234567890

Date: March 16, 2026